

chatHealthy.ai

Business Plan

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chatHealthy.ai is building the infrastructure for informed consumer healthcare decisions — giving individuals the one thing the healthcare industrial complex has systematically withheld: the ability to know the quality of care available to them, within their insurance, their location, and their circumstances. This is a long-duration, pre-revenue venture requiring patient capital and shared vision. The opportunity is foundational.

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1. Vision

chatHealthy.ai is a consumer healthcare navigation platform that uses artificial intelligence to tell individuals not just where they can seek reimbursable care — but which of their available options delivers the highest quality care. It does this using only publicly available data, assembled and reasoned over by an agentic AI system that understands each consumer's unique context: their insurance, location, access to transportation, and health situation.

This is disruptive by design. Insurance companies, healthcare providers, and pharmaceutical companies have a structural incentive to keep both quality and cost information opaque. Consumers today make some of the most consequential decisions of their lives — which doctor to see, which hospital to use, which treatment path to pursue — with almost no actionable information about what that care will cost them or how good it actually is. chatHealthy.ai changes that.

The analogy is not incremental. Facebook democratized social connection. Amazon democratized commerce. Waze gave drivers information that road authorities and mapping incumbents didn't want to give them. chatHealthy.ai intends to do the same for healthcare navigation — building the consumer trust and network that makes it indispensable before the business model is fully defined. The monetization follows the trust.

2. The Problem

What Consumers Face Today

A consumer who needs care today faces a system designed to obscure rather than illuminate. They have an insurance card, a phone number, and a provider directory that is frequently out of date, organized by specialty rather than quality, and offers no indication of which providers produce better outcomes. They do not know which covered cardiologist in their network has the best survival rates. They do not know which covered hospital has the lowest readmission rates for their condition. They do not know if the specialist their primary care doctor referred them to is in-network, accepts new patients, or is even still practicing.

For uninsured consumers the problem is compounded: they do not know what assistance programs exist, what care is available to them at reduced or no cost, or how to access the system at all.

Why This Problem Persists

- Insurers profit from complexity — an informed consumer shops more effectively, challenges more denials, and demands more from their network
- Providers resist quality and cost transparency — rankings create accountability and competitive pressure that the current system insulates them from
- Pharma benefits from consumer confusion about treatment options, formulary coverage, and comparative drug costs
- Existing tools (Zocdoc, Healthgrades, insurance portals) are either incomplete, biased toward paid placement, or deliberately avoid integrating quality and cost with coverage and access

3. The Solution

What chatHealthy.ai Does

chatHealthy.ai is an AI-powered healthcare navigation assistant. Given a consumer's individual context — insurance plan, location, transportation access, and health situation (via uploaded health records or an AI-guided interview) — the system identifies covered, accessible care options and ranks them by quality using publicly available data.

Core Features at Alpha Launch (May 1, 2026)

- Health insurance network navigation — find covered providers by specialty and location
- Provider directory management — current, accurate, searchable
- Emergency triage safety check — detects consumers who need emergency care and routes them to emergency services immediately; refuses to engage as an advisor in those situations
- Explicit scope boundary — the system will not give clinical advice under any circumstance; its role is navigation, not diagnosis
- Uninsured consumer support — routes uninsured users to available assistance programs and accessible care

Alpha features are free and stateless. No accounts required beyond optional email and phone for follow-up contact, collected only with explicit user consent.

The Killer App — Quality Transparency (Beta and Beyond)

The feature that makes chatHealthy.ai genuinely disruptive is quality and cost-ranked care recommendations. Using publicly available data sources — CMS quality measures, hospital compare data, physician quality reporting, state health department data, Medicare cost data, and others — the

system will surface not just where a consumer can get covered care, but which of their options is demonstrably better and what it is likely to cost them.

This combination — quality and cost, integrated with coverage, access, and individual consumer context — has never been assembled into a consumer-facing tool. That is the gap chatHealthy.ai fills.

4. Technology Architecture

chatHealthy.ai is built on an end-to-end agentic AI system. At its core is a multi-model 'brain' that orchestrates collaboration between Claude (Anthropic) and GPT (OpenAI), with context stored and managed in MongoDB. The architecture is designed from the ground up for autonomous operation — the current man-in-the-middle phase is a validation step, not the end state.

- Claude (Anthropic) handles complex reasoning, quality assessment, and nuanced healthcare navigation logic
- GPT (OpenAI) collaborates on complementary tasks within the multi-model brain
- MongoDB provides persistent, session-aware context management across interactions
- Agentic infrastructure enables the system to scale to fully autonomous operation without rebuilding the core

Data Privacy and Safety Architecture — Built In from Day One

- No identified health data collected at alpha — only email and phone, with explicit user consent
- Emergency triage detection — consumers showing signs of needing emergency care are routed to emergency services, not engaged as advisory cases
- Hard clinical advice boundary — enforced at the system level, not just policy
- Usage policy clearly defines what the system is and is not; users acknowledge scope before engaging
- Minimal data collection by design — avoids HIPAA covered entity complexity at alpha while building a trust-first relationship with consumers

These are not afterthoughts. Building safety and privacy into the architecture from the start is both the ethical choice and the strategically correct one — it reduces regulatory risk, builds consumer trust, and differentiates chatHealthy.ai from tools that treat compliance as a retrofit.

5. Market Opportunity

The United States healthcare system represents approximately \$4.5 trillion in annual spending. Consumers are the decision-makers at the center of that system yet are the least informed participants in it. Every American with health insurance — approximately 300 million people — is a potential user of chatHealthy.ai. Every uninsured American is equally a potential user.

The market analogy that best fits is not healthcare software — it is consumer infrastructure. Waze did not sell navigation software; it built a network that made every driver more informed. chatHealthy.ai is building the equivalent network for healthcare consumers. The value compounds with scale.

Why Now

- AI capability has reached the point where a single founder with the right architecture can build what previously required a team of hundreds
- Public healthcare quality data has expanded significantly — CMS, state health departments, and other sources now publish more machine-readable quality data than ever before
- Consumer distrust of the healthcare system is at an all-time high — the audience is primed for a tool that is explicitly on their side
- The agentic AI moment means the system can reason over complex, multi-variable consumer situations in real time — this was not feasible even two years ago

6. Competitive Landscape

Existing tools address fragments of the problem but none integrate coverage, access, and quality into a personalized consumer view:

- **Zocdoc** — appointment booking; no quality data; biased toward paid provider placement
- **Healthgrades / Vitals** — provider reviews and basic quality signals; not integrated with insurance coverage or consumer context
- **Insurance company portals** — show in-network providers but actively avoid quality transparency; designed to minimize claims, not optimize consumer outcomes
- **CMS tools (Care Compare)** — comprehensive quality data but not consumer-friendly, not personalized, not integrated with insurance
- **Hospital marketing tools** — exist to drive volume to specific systems, not to serve consumer interests

chatHealthy.ai's defensible position is the integration layer — combining coverage, access, context, and quality in a single AI-native consumer experience. No incumbent has the incentive to build this. That is the opportunity.

7. Founder

Skip Snow is a senior technology and architecture executive with over 25 years of experience at the intersection of enterprise technology and healthcare. His background spans every sector relevant to chatHealthy.ai — payer, provider, health IT, research, and startup — and he has operated at the most senior levels of organizations that define the system he is now working to disrupt.

Directly Relevant Experience

- **Humana (2022–2026)** — Enterprise Architect at a \$117B healthcare organization; re-architected IAM infrastructure; led enterprise-wide data quality initiatives across Medicare, Commercial, and Specialty lines
- **Anthem (2020–2022)** — Director, Solution Management; directed a 60-person organization eliminating \$1.2B of compliance risk; contributed to enterprise AI strategy
- **Prospect Medical Holdings (2019)** — Acting Chief Architect for a \$3B healthcare system spanning hospitals, a care management company, and a health insurance company across 5 states
- **Kaiser Permanente (2007–2009)** — VP, Technology Architecture; managed care delivery, infrastructure, and security architecture; formulated architecture strategy for a multi-billion-dollar infrastructure upgrade
- **Forrester Research (2013–2015)** — Principal Analyst, Healthcare Software; published syndicated research on AI, telehealth, big data, and population health; helped triple healthcare research revenue
- **Citigroup (2004–2007)** — SVP Enterprise Architecture; introduced machine learning to a global KYC database project; chaired the Architecture Round Table
- **HIPAA Box (2009–2019)** — CEO and Founder; raised \$275K; built a team of 20; won hospital and clinic market mind share; ultimately failed due to a misjudged storage market and over-focus on technology innovation. Lessons internalized.

Why This Founder for This Problem

Skip Snow has sat inside the payer system at Humana and Anthem. He has run the architecture of a multi-state provider system at Prospect. He has written the research on healthcare AI at Forrester. He has built and failed a healthcare startup. He understands — from the inside — why the system withholds quality information from consumers, how the data is structured, where it is publicly available, and what it would take to assemble it into something consumers can use.

He is also an outsider artist — oil on linen, exhibited work, a different kind of thinker. He was a studio assistant to Jean-Michel Basquiat and other well-known artists. The founder of a company built to disrupt a system that resists transparency should probably not think like the system does.

8. Operational Model — One Person, Agentic Scale

chatHealthy.ai is intentionally designed as a one-person company with an agentic operational footprint. This is not a limitation — it is a structural competitive advantage and a proof of concept for what the AI moment makes possible.

The traditional assumption in software is that scale requires headcount. chatHealthy.ai challenges that assumption directly. An agentic AI infrastructure handles tasks that would previously have required engineering teams, compliance departments, research analysts, and operations staff. The founder architects, supervises, and directs — the agents execute.

What This Means in Practice

- Development velocity: agentic coding infrastructure (Claude Code) compresses build timelines that would previously require a full engineering team
- Regulatory research: agentic research capability tracks and encodes compliance requirements across all jurisdictions without a dedicated compliance function
- Data assembly: AI-powered research and synthesis replaces the analyst teams that incumbents rely on to aggregate quality and cost data
- Operations: minimal overhead, no office, no management layers — capital goes directly into product and infrastructure

Why This Is a Competitive Advantage

Large incumbents — insurance companies, hospital systems, health IT vendors — carry enormous operational overhead. Their compliance costs alone dwarf chatHealthy.ai's entire operating budget. Their ability to move quickly is constrained by organizational complexity that chatHealthy.ai does not

have and does not need.

The lean agentic model means chatHealthy.ai can iterate faster, respond to regulatory changes faster, and deploy capital more efficiently than any incumbent in the space. It also means that every dollar of runway goes further — extending the time available to build the network effect that makes the platform indispensable.

The one-person agentic company is itself a demonstration of the thesis: AI makes it possible for a single informed human to do what previously required an organization. chatHealthy.ai is building that capability for consumers in healthcare navigation.

9. Funding Strategy

chatHealthy.ai is not seeking early revenue. It is seeking runway. The companies that have become pillars of the consumer internet — Amazon, Facebook, Google, Waze — were not built around early monetization. They were built around indispensability. chatHealthy.ai is pursuing the same path: build something consumers cannot imagine living without, then monetize from a position of trust and scale.

Near-Term Funding Approach

- Self-funded through alpha launch (May 1, 2026) — no external capital required to reach first users
- Professional services revenue to sustain operations during the build phase
- Non-dilutive credits via Anthropic Startup Program (target: claude.com/programs/startups) — API credits and rate limits without equity
- Equity only when valuation reflects demonstrated execution — not before

What We Are Looking For in Investors

We are not looking for investors who need a 3-year revenue model. We are looking for investors who understand that the most valuable consumer platforms in history required patient capital and a willingness to let the network effect compound before extracting value. The monetization options for a trusted, scaled consumer healthcare platform are significant and varied — but they are a second-order conversation. The first-order conversation is: do you believe consumers deserve to know the quality of care available to them? If yes, this is the investment.

Potential Monetization Paths (Future)

These are illustrative, not commitments. The right model will emerge from how consumers use the platform.

- B2C subscription for premium features (deeper quality analysis, ongoing care navigation)
- Employer benefits integration — offer chatHealthy.ai as a navigation benefit
- Healthcare institutional licensing — health systems, ACOs, and payers seeking consumer engagement tools
- Data and insights licensing — aggregated, de-identified consumer navigation patterns
- Advertising from providers who want to be found — on consumer terms, not provider terms

10. Roadmap

Alpha — May 1, 2026

- Consumer-facing web application
- Health insurance network navigation and provider directory
- Emergency triage safety check
- Uninsured consumer routing
- Free, stateless features — no accounts required
- Email and phone collection for interested users (explicit consent only)

Beta

- Quality-ranked care recommendations — the killer app feature
- Consumer context intake — insurance, location, transportation, health situation
- Chart upload and AI-guided health interview
- Expanded publicly available quality data integration

Production 1.0

- Full personalized care navigation
- Ongoing care relationship — not just point-in-time search
- Institutional features for health systems and payers
- API access for employer benefits integration

11. Regulatory Positioning

chatHealthy.ai is designed to be compliant with every applicable state and federal regulatory framework — not just California. This is a strategic requirement: the long-term vision includes selling to state health plans, hospital systems, and institutional partners across all fifty states. Partial compliance is not a viable foundation for that ambition.

Compliance as an Agentic Capability

What makes this achievable for a one-person company is the same thing that makes chatHealthy.ai's operational model a competitive advantage: agentic AI research. Regulatory compliance in healthcare is traditionally expensive because it requires armies of lawyers and compliance professionals to parse dense, frequently changing state and federal law. chatHealthy.ai encodes regulatory research and monitoring directly into its agentic infrastructure.

With human supervision and AI-powered legal and regulatory research, the company can track and encode applicable requirements across all jurisdictions as a routine part of the build process — not as a separate compliance function requiring dedicated headcount. This is not a workaround; it is a structural advantage that incumbents with large compliance organizations cannot easily replicate.

Current Regulatory Architecture

- Hard boundary against clinical advice keeps the product outside FDA software-as-a-medical-device (SaMD) territory — enforced at the system level, not just policy
- Minimal data collection (email and phone only, explicit consent, no identified health data) avoids HIPAA covered entity complexity at alpha
- Any future collection or processing of protected health information will trigger HIPAA obligations — architecture will be updated accordingly before that threshold is crossed
- Emergency triage safety feature is documented and defensible — critical for compliance review and investor due diligence
- State-level regulations — California CMIA, state telehealth laws, and equivalents in all target states — are tracked and encoded via agentic research capability
- National compliance posture is a requirement, not an aspiration — built into the roadmap from day one

This section does not constitute legal advice. Human legal review remains part of the compliance process — the agentic capability augments and accelerates that review, it does not replace professional judgment on high-stakes regulatory questions.

12. Anthropic Partnership Target

chatHealthy.ai is built on Claude (Anthropic) as a core component of its agentic brain. We are actively pursuing Anthropic's startup support programs as a source of non-dilutive API credits and rate limit access.

Option 1 — Anthropic Startup Program (Primary Target)

- URL: claude.com/programs/startups
- Provides: API credits, highest publicly available rate limits, founder resources
- Cost: No equity — a biz dev and partner program
- Requirement: Referral through an active Anthropic partner VC firm

Option 2 — Anthology Fund (Deferred)

- URL: menlovc.com/anthology-fund
- A \$100M venture fund — partnership between Menlo Ventures and Anthropic
- Invests seed to expansion; includes \$25K in Anthropic API credits
- Healthcare AI is an explicit focus area
- This involves equity — deferred until valuation reflects demonstrated execution

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